



# Capacity Management and Escalation

Reference #: FSFH-HW-POL-0058

| Service                                      | Service/Department/Unit | Disciplines                                                  |
|----------------------------------------------|-------------------------|--------------------------------------------------------------|
| Fiona Stanley Hospital<br>Fremantle Hospital | Hospital Wide           | Medical, Nursing, Allied Health<br>and Facilities management |

## 1. Introduction

The Fiona Stanley Fremantle Hospital Group (FSFHG) Capacity Management and Escalation Policy is designed to establish a consistent and steady flow of patients through Fiona Stanley and Fremantle Hospitals. This is crucial to ensure optimum bed availability in order to accommodate fluctuations in workload effectively.

This policy aims to ensure that every emergency admission is allocated an appropriate bed within four hours, and that no elective admissions are cancelled due to lack of bed availability. The policy outlines how this process will be managed whilst maintaining high standards of patient safety, patient experience and performance against key waiting times and quality standards. The purpose of this policy is to ensure that:

“All patients are admitted to the right place at the right time, first time”

In order to achieve this, the escalation policy will assist towards the following:

- Early identification of capacity problems
- Proactive rather than reactive management
- Clear and concise actions
- Defined responsibilities.
- Having an escalating cascade of actions balancing the varying risks across areas

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## 2. Terminology

|                                     |                                                                                                                                                                                                                                                                                                                                                              |
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| <b>ED Safety Notification Alert</b> | When the Emergency Department has more patients than it can potentially safely care for.                                                                                                                                                                                                                                                                     |
| <b>Red Square Patient</b>           | A ward patient who is for discharge on that day who is placed in the ward/unit corridor to accommodate a patient admission from ED or a suitable patient from ED/AMU/ASU who can be safely placed whilst admission bed is sourced on ward                                                                                                                    |
| <b>Red2Green</b>                    | A visual management system that identifies lost time (red days) in a patient's journey.                                                                                                                                                                                                                                                                      |
| <b>SAFER Principles</b>             | A blend of five elements used as a tool to reduce delays for patients in inpatient wards.                                                                                                                                                                                                                                                                    |
| <b>Over Census Bed</b>              | A bed that is over the opened number of funded beds on that ward.                                                                                                                                                                                                                                                                                            |
| <b>CHOICE</b>                       | Consider Home Over Inpatient Care Every time (CHOICE) - A philosophy focused on ensuring the earliest possible discharge in recognition of the risks associated with hospital admissions in weight against the potential risks associated with discharge. It is supported by a discharge to assess model and clinical support team for the home environment. |

## 3. Policy

### 3.1. Hospital Escalation Status

The hospital bed capacity status has 4 levels which are outlined in the plan below and contain descriptors of scenarios, which can trigger the hospital wide response. In each level, a series of actions will be implemented by key staff members to ensure that risk to patients is minimized. The overall hospital capacity status will be determined by a combination of:

- Bed capacity
- Emergency Department (ED), activity, capacity, acuity and/or exit block, surge
- Triggers affecting certain services.

These levels are:

- BRAG Status GREEN (normal working conditions)
- BRAG Status AMBER (early signs of difficulty, some excess pressure requiring extra management)
- BRAG Status RED (persistent excess pressure requiring significant additional management action)
- BRAG Status BLACK (Severe and/or prolonged excess pressure requiring support from external agencies).

The organizations BRAG status is regularly reviewed and targeted communication across the site via base pages, pop-up messages is utilized. There is a capacity situation report email at 0800, 1430 and 2000 by the Capacity and Access Service (CAS).

The service escalation sub-plans for the areas where patient flow delays impact on the continuation of work in that area can be found on the Hub including the [SMHS Mental Health Patient Flow Management Plan](#) and the [FSH Maternity Services - Obstetric Bypass Triggers and Implementation Policy and Procedure](#) and will be activated if patient flow delays meet the criteria for the sub-plan

Key staff roles and action cards for the four Hospital bed capacity escalation levels are included in Appendix 1 and 2 describing the actions expected at the respective BRAG status.

### **Each Brag status has a corresponding action card for each service to follow.**

### **3.1. FSFHG Capacity BRAG Escalation Indicators**

| Green                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | Amber                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | Red                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | Black                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
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| <ul style="list-style-type: none"> <li>&gt;34 general inpatient beds available (&lt;92% capacity) indicating sufficient capacity for next 24 hours</li> <li>AMU has sufficient space to accommodate direct admissions and referred patients from ED</li> <li>ED has sufficient capacity (&gt;8 beds free combined in Resus and Assessment)</li> <li>No cancellation of elective work as a result of bed pressures</li> <li>ICU ward transfers can be accommodated</li> <li>Direct admissions and external transfers are able to be accepted</li> </ul> | <p><b>Priority 1 Indicators - Any one trigger</b></p> <ul style="list-style-type: none"> <li>34-22 general inpatient beds available for admitting patients (92-95% capacity)</li> <li>AMU approaching total capacity with few discharges or transfers pending</li> <li>Projected emergency admissions exceed predicted discharges</li> </ul> <p><b>Priority 2 Indicators - Any two triggers</b></p> <ul style="list-style-type: none"> <li>No ICU capacity with delays for ward transfers out</li> <li>ED has &lt;8 beds free in Resus/Assessment</li> <li>No imminent resolution to staffing shortfalls in clinical areas</li> </ul> | <p><b>Priority 1 Indicators - Any one trigger</b></p> <ul style="list-style-type: none"> <li>21-9 general inpatient beds available (95-98% capacity) indicating a deficiency in medical/surgical beds over the 24 hour period</li> <li>AMU full</li> <li>Emergency admissions are exceeding declared discharge levels</li> <li>Potential need to cancel surgery for same or next day</li> </ul> <p><b>Priority 2 Indicators - Any two triggers</b></p> <ul style="list-style-type: none"> <li>No ICU capacity with delays for ward transfers out</li> <li>All ED bays occupied with one patient on A Bay</li> <li>Ward closures due to infection control affecting patient flow</li> <li>Staffing issues with potential to affect care not resolved in key areas</li> </ul> | <p><b>Priority 1 Indicators - Any one trigger</b></p> <ul style="list-style-type: none"> <li>&lt; 9 beds available for admitting patients (&gt;98% capacity)</li> <li>All ED bays occupied with 1 patient on Ramp</li> <li>Adult surgical cancellations on the day for ward capacity reasons</li> <li>Bed state indicates a sustained deficiency in medical beds with no immediate space in AMU</li> </ul> <p><b>Priority 2 Indicators - Any two triggers</b></p> <ul style="list-style-type: none"> <li>No ICU capacity and no patients identified to transfer out</li> <li>No beds available to outlie medical patients into</li> <li>No further discharges expected</li> </ul> |

### 3.2. Capacity Code Yellow

A Code Yellow should be considered where the ED and hospital are both BRAG Black and there is a predicted decline in site status which could compromise patient safety and organizational function. Escalation to Code Yellow will be authorized at executive level only and a pop-up and paging alert sent out to inform staff at both Fiona Stanley Hospital (FSH) and Fremantle Hospital (FH) .

The Capacity Code Yellow will be coordinated by the Nurse Director CAS with the executive team meeting frequently . See Appendix 3 for Code Yellow Action Card.

### 3.3. Emergency Department BRAG Capacity Escalation

The FSFHG is required to meet the Western Australian Emergency Access Time (WEAT) target of 90%, whereby ED attendees have their journey completed by either being discharged, transferred or admitted to a hospital bed within four hours of arrival. Breaches to this standard are monitored by the ED Nurse Unit Manager (NUM), Associate Nurse Unit Managers(ANUMs)/Shift Coordinator, CAS Access Coordinators, Hospital Coordinators and ward on an ongoing basis throughout the shift, in liaison with the ED Medical Coordinator.

The ED has a set of timelines regarding the patient journey in the ED to assist in maintaining ED capacity (see Table 1) and a traffic light BRAG response to accurately account for ED activity and status with a set of hospital responses to assist with resolving ED capacity (see Table 2). As the hospital has competing priorities ED functions under a 'push' model to ensure suitable patients are transferred to the appropriate units ensuring capacity for new emergency presentations.

**Table 1. ED Patient Timelines**

|                                                                                                                                                                                                                                                                                                                                                                                                                           |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Inclusion Criteria:</b> <ul style="list-style-type: none"> <li>• All patients with an ADDS 0-3</li> <li>• Bed available on the receiving ward.</li> </ul> <b>Exclusion Criteria</b> <ul style="list-style-type: none"> <li>• ADDS&gt;4</li> <li>• Risk of compromise to airway</li> <li>• Likely discharge from the Emergency Department</li> <li>• Patients requiring additional mental health assessment.</li> </ul> |
| <b>0 – 2 hours</b>                                                                                                                                                                                                                                                                                                                                                                                                        |
| <ul style="list-style-type: none"> <li>• Decision to admit made by the ED Medical Staff</li> <li>• Booking request entered on EBM and patient referred to Specialty Team</li> <li>• Time of Referral, Name of Specialty Dr, ADDS documented by ED Medical Staff in EBM.</li> </ul>                                                                                                                                        |
| <b>30 minutes from time of referral</b>                                                                                                                                                                                                                                                                                                                                                                                   |
| Speciality accepts patient and patient will be reviewed on the ward as per IPS <ul style="list-style-type: none"> <li>• Clinical handover and patient sent to the ward</li> <li>• Document delays on EDIS</li> </ul> <b>or</b> <ul style="list-style-type: none"> <li>• Speciality request to review in ED, prior to admission or to potentially discharge.</li> </ul>                                                    |
| <b>&gt; 30 minutes from time of referral</b>                                                                                                                                                                                                                                                                                                                                                                              |

- ☐ Specialty did not attend within 30 minutes, specialty assumes acceptance of care as per IPS
- ☐ ED Senior Medical Dr informs accepting Specialty the patient is going to the ward
- ☐ Escalate to CAS for identification of ward bed allocation
- ☐ Clinical handover and patient transfer to the ward
- ☐ Document updates on EDIS.

Patients transferring to the ward without Specialty review must have:

- ☐ Senior sign off by ED Senior Medical Dr
- ☐ Documented 6 hour plan
- ☐ Medication Chart with IV fluids if appropriate ward appropriate analgesia prescribed
- ☐ Recent ADDS 0-3 performed within 30 minutes

Once the above is completed, ANUM/Area lead reviews and ticks "ready to leave" on EBM  
(Pt turns purple on EBM)

## Internal Professional Standards

Internal Professional Standards (IPS) denote the escalation points in each part of the patient's journey and detail the actions to be taken to identify and unblock any delays in the pathway. ED Area Medical Lead or ED Medical Co-ordinator are responsible for escalating admission delays/disputes with Inpatient team Consultant. Specialty teams are required to ensure that patients referred to them by ED are seen within the standard guidelines. Patient referral by ED, within the guidelines of the agreed unit IPS, is a one-way process and on-referral is the responsibility of the inpatient team (AMU excluded).

### Table 2. FSH ED Capacity Status Triggers

SHIMS auto-updates on capacity and surge information directly from EDIS. ED status is measured separately from the hospital status since the two are interdependent. The ED BRAG status is triggered when any **one** of the three triggers of Surge, Acuity or Exit Block are met.

The ED Associate Nurse Unit Manager overseeing ED flow will review the departments BRAG status every four hours, inform CAS and update SHIMS regarding acuity triggers. CAS will send out a page to alert the action holders of the ED BRAG status.

| ED BRAG: GREEN                                                                                                                                                                                                                                                                                                                                              |                                                                                                                                                                                                                                                                                                                                                                                                      |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Trigger Criteria                                                                                                                                                                                                                                                                                                                                            | Simultaneous Actions                                                                                                                                                                                                                                                                                                                                                                                 |
| <p><b>Any of the below</b></p> <p><b>Surge</b><br/>           ≤20 patients in 2 hours<br/>           OR<br/>           ≥8 beds free combined in Resus + Assessment</p> <p><b>Acuity</b><br/>           All P1 patients seen &gt;1hr ago.</p> <p><b>Exit Block</b><br/>           &lt;5 patients with EBM booking slip in excluding those pulled to ESSU</p> | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Business as usual, each pod runs its own flow. ED ANUM reviews department BRAG status 4 hourly.</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Adhere to hospital flow policy and medical Internal Professional Standards.</li> </ul> |

| ED BRAG: AMBER                                                                                                                                                                                                                                                   |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
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| Trigger Criteria                                                                                                                                                                                                                                                 | Actions as above plus (to occur within 30mins unless otherwise indicated):                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
| <p><b>Any of the below</b></p> <p><b>Surge</b><br/>           &gt;20 patients in 2 hours<br/>           OR<br/>           &lt;10 beds free combined in Resus + Assessment</p>                                                                                    | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>Medical coordinator to review all waiting patients to assess if any can be restreamed to MDAC &amp; AECC</li> <li>Medical coordinator ensures ESSU is full</li> <li>ANUM and Medical Coordinator to undertake medical round to assess suitability for patients to be transferred to waiting room, transit lounge, ESSU or AECC</li> <li>Area Leads to undertake clinical rounding/huddles to confirm plans and utilize 'fit to sit' for appropriate patients</li> <li>Load level between areas, redeploy staff to area of longest wait to be seen</li> <li>Non clinical EDANUMs come to floor and assist in area assessments/ escalate flow issues</li> <li>Fit2Sit review of patients in MDAC with these patients moved to sub-wait/waiting room</li> <li>Discharges moved to fit to sit areas</li> <li>EDANUM to ensure that transit lounge suitable patients are easily identified in EBM</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>CAS to allocate any patient waiting longer than 30 minutes to an available specialty bed on EBM.</li> <li>CAS to allocate specialty bed slips to home ward from admission units.</li> <li>Ensures that hospital Brag status actions supports ED actual and potential admissions</li> <li>CAS to transfer 4 patients (ADDS &gt;3) to transit lounge</li> </ul> |
| <p><b>Acuity</b><br/>           1 new P1 (&lt;1hr ago)<br/>           OR<br/>           ≥2 critical care patients in Resus<br/>           OR<br/>           Surge of P2 patients in Resus i.e.<br/>           &gt;3 P2 patients without medical 'time seen'.</p> | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Activate Priority 1 team response</li> <li><input type="checkbox"/> Escalate any ICU/CCU/HACA referrals to Resus Consultant if &gt;30 minutes for review</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> ICU team to collect pulled patient from ED.</li> <li><input type="checkbox"/> CAS to allocate ICU patient once ICU confirmed acceptance and ensure patient is transferred immediately when clinically appropriate</li> <li><input type="checkbox"/> If CAS anticipate any non-clinical delay in the transfer of the patient to ICU exceeding 30 mins, CAS to escalate to ND CAS or HC out of hours.</li> <li><input type="checkbox"/> AMU consultant to review current HACA patients to ensure HACA is able to receive additional admissions</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| <p><b>Exit Block</b><br/>           5-15 patients with EBM booking slip in excluding those pulled to ESSU</p>                                                                                                                                                    | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>Medical area leads to complete 6-hour plans, and push stable patients to ward at 30 minutes post referral</li> <li>Transfer 4 patients (ADDS &gt;3) to transit lounge</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>CAS to review, in liaison with ED ANUM, all ED patients at the 2.5-hour mark regarding plans and update EDIS/EBM comments</li> <li>Wards to pull appropriate patients to bed if bed clean has commenced</li> <li>PLN and MH Consultant to ensure disposition decisions on all MH patients by midday</li> <li>Escalation to ND CAS/Service 5 of any mental health patients &gt; 4 hours with no admission plan</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |



| ED BRAG: RED                                                                                                                                                                                                                    |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
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| Trigger Criteria                                                                                                                                                                                                                | Actions as above plus (to occur within 30mins unless otherwise indicated):                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <p><b>Any of the below</b></p> <p><b>Surge</b><br/>           &gt;40 patients in 2 hours<br/>           OR<br/>           1 patient on A Bay</p>                                                                                | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>Medical Coordinator/ANUM to review SJA distribution with SJA Hospital Liaison Manager (HLM) and escalate to ADC if perceived uneven</li> <li>ED Allied Health Team to expedite ESSU reviews and undertake ED round to generate referrals early</li> <li>ED ANUM to liaise with orderly manager to request surge portering</li> <li>Medical coordinator to confirm status and inform CAS of an ED BRAG Red – Surge and/or Acuity and/or Exit block</li> <li>ED NUM and second ANUM to cease non-clinical duties and provide support and escalate issues to their Nurse director</li> <li>ED prepare for potential to complete ED safety notification</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>Inpatient teams to prioritise review of patients in the ED</li> <li>CAS to review, in liaison with ED ANUM, all ED patients at the 2.5-hour mark regarding plans and update EDIS/EBM comments</li> <li>Surge orderlies to attend ED to assist with urgent patient movement</li> </ul> |
| <p><b>Acuity</b><br/>           2 new P1s (&lt;1hr ago)<br/>           OR<br/>           ≥3 critical care patients in Resus<br/>           OR<br/>           Inability to accommodate a P2 patient in an appropriate ED bed</p> | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> ED Resus Medical Lead escalate to ICU consultant for immediate assistance with or review of high acuity patients</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> After Hours: ALERT CNS to attend ED to assist with care of high acuity patients</li> <li><input type="checkbox"/> Hospital Co-ordinator to instruct ward co-ordinators to arrange patient pick-up from ED by ward nurses</li> <li><input type="checkbox"/> ICU consultant to determine best option for immediate ED support</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                 |
| <p><b>Exit Block</b><br/>           15-19 patients with EBM booking slip in excluding those pulled to ESSU</p>                                                                                                                  | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>ED ANUM to liaise with orderly manager to request extra orderlies and cleaners for ED</li> <li>ESSU Consultant to undertake additional ESSU round to expedite discharges</li> <li>ED Medical Lead to identify ESSU patients that can convert to AMU/AECC</li> <li>ED Medical coordinator to identify patients with an ADDS 3 or under (maximum 1 per admitting ward) and ensure 6-hour plans are in place for anticipation of FULL CAPACITY PROTOCOL being enacted</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>ED to transfer a further 4 patients awaiting admission to transit lounge (0800-1400)</li> <li>FULL CAPACITY PROTOCOL enacted by CAS</li> <li>Transfer patients awaiting admission to 'red square' for ALL FSH B block multi-stay wards</li> </ul>                                                                                                                                                                                                                      |



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|  | <ul style="list-style-type: none"> <li><input type="checkbox"/> Hospital Co-ordinator to interrogate EBM for bed requests not allocated to known hospital capacity and organize best-fit allocation and update EBM.</li> <li><input type="checkbox"/> CAS to direct NUMs/Shift Coordinators to move patients into over-census beds</li> <li><input type="checkbox"/> Service Consultants/NUMs to review need for admission for direct medical/surgical admissions</li> <li><input type="checkbox"/> One voluntary specialty accepted for admission FSH mental health patient to move to communal mental health area within each mental health ward during the hours 0800-1600 following discussion with NUM and /or HOOT 4 MH.</li> <li><input type="checkbox"/> AMU/Geriatric teams to identify if any ED admissions can go direct to AECC or FH</li> <li><input type="checkbox"/> Orderlies and cleaning supervisor to allocate staff to assist with urgent patient movement</li> <li><input type="checkbox"/> Ensure FH beds are occupied</li> <li><input type="checkbox"/> ED MH team, if otherwise untasked, to commence assessments on known MH patients prior to ED medicalreview</li> </ul> |
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| ED BRAG: BLACK                                                                                                                                                                                                                        |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
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| Trigger Criteria                                                                                                                                                                                                                      | Actions as above plus (to occur within 30mins unless otherwise indicated):                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
| <p><b>Any of the below</b></p> <p><b>Surge</b><br/>           &gt;90 patients in ED<br/>           OR<br/>           1 patient on Ramp</p>                                                                                            | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> ED to complete ED Safety Notification (EDSN)</li> <li><input type="checkbox"/> Night shift: Call and update on-call ED Consultant</li> <li><input type="checkbox"/> Children's ED to take over any 17 year old patients</li> <li><input type="checkbox"/> ED to identify 2 patients to be moved to AECC/ASU hot clinic</li> <li><input type="checkbox"/> Discuss with ANUM/ED nurse coordinator about resources available to turn MDAC into an inpatient holding zone.</li> <li><input type="checkbox"/> ASP/NPs to relocate from MDAC to pull appropriate ambulatory patients direct from the waiting room into an alternative space</li> <li><input type="checkbox"/> Medical coordinator, VEM or RAT lead to move to waiting room and identify patients with potentially high clinical risk speak to relevant Priority 4 and 5 ambulatory patients of the expected prolonged wait and provide alternative care options should they not wish to wait</li> <li><input type="checkbox"/> ESSU to release nurse to support WR/Ramp</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Director Allied Health to send an additional OT, PT and SW to ED to assist with assessing and facilitating discharge of patients</li> <li><input type="checkbox"/> CAS to arrange for an AMU/ASU/CCU or ICU nurse to attend ED.</li> <li><input type="checkbox"/> ED MH team, if otherwise untasked, to commence assessments on known MH patients prior to ED medical review.</li> <li><input type="checkbox"/> Radiology consultants to support ED CT reporting as a priority to facilitate ED patient flow and disposition decisions</li> <li><input type="checkbox"/> Medical and Nursing Director on call document response to EDSN Recommendations within 1hr of EDSN</li> <li><input type="checkbox"/> CAS informs the HIC and ensures a 'pop up' alert occurs within 10 minutes</li> <li><input type="checkbox"/> All hospital executive (including on-call out of hours) notified via text message of ED Safety Notification and BRAG status Black</li> <li><input type="checkbox"/> Key hospital executive, as indicated by Safety Notification, to complete 'closed-loop responses' for EDSN recommendations.</li> </ul> |
| <p><b>Acuity</b><br/>           3 new P1s (&lt;1hr ago)<br/>           OR<br/>           ≥4 critical care patients in Resus<br/>           OR<br/>           Unable to decant resus to immediately absorb 2 priority one patients</p> | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>• Medical Coordinator and ANUM to contact theatres about taking over any planned procedures, and cardiology team to take over planned cardioversions</li> <li>• Medical Coordinator ensures all GPs who call to discuss an ED arrival are informed of expected wait times</li> <li>• Medical Coordinator calls ICU consultant and/or Duty Anaesthetist to request immediate assistance in ED Resus.</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>• Service Directors 1,2&amp;3 discuss with ICU/CCU or HACA Consultants to attend ED to see accepted patients</li> <li>• Service 2 Nurse Director or Hospital Co-ordinator (out of hours) to arrange an ICU nurse to attend ED to care for a high acuity patient until an appropriate bed becomes available</li> <li>• Cardiology to attend ED and, following liaison with ED medical coordinator, commence assessment of cardiology patients</li> <li>• Medical and Nursing Director on call document response to EDSN Recommendations within 1hr of EDSN.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |

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| <p><b>Exit Block</b><br/>         ≥20-24 patients with EBM booking slip in excluding those pulled to ESSU</p> | <p><b>Emergency Department</b></p> <ul style="list-style-type: none"> <li>• ED admitted patients to be identified for transfer to AECC or Surgical Hot Clinic</li> <li>• Discuss with ANUM/ED nurse coordinator about resources available to turn MDAC into an inpatient holding zone.</li> <li>• ED to review patients with &gt;24hours in assessment and short stay area to determine discharge or specialty bed</li> <li>• ED to utilize ESSU red square for a patient in Resus/Assessment with ESSU booking slip</li> </ul> <p><b>Hospital Response</b></p> <ul style="list-style-type: none"> <li>• AMU and ASU registrars to attend ED to complete 6-hour plans, and ADDS modification orders for EBM listed patients, and liaise with Medical Coordinator to see new patients likely to be referred to their unit including those likely to be dischargeable following their review.</li> <li>• Wards may choose to reallocate an existing patient into the 'red square' to allow the ED patient access to a room</li> <li>• Ward Coordinators to identify ward patient to transfer to redsquare when morning shift commence.</li> <li>• All alternative streams of hospital admission require case by case discussion and approval from DCS (otherwise have their admission cancelled)</li> </ul> |
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Figure 1. Emergency Department Safety Alert and Full Capacity Protocol

| Emergency Department – Safety Alert                                                                         |                                                                    |                                                                                   |
|-------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| <b>Trigger</b><br>Any ATS 1 or 2 patients unable to access Resus                                            | <b>Trigger</b><br>Any patient on the ramp for more than 60 minutes | <b>Trigger</b><br>Any ATS 3 patients unseen in waiting room for more than 3 hours |
|                                                                                                             |                                                                    |                                                                                   |
| <b>Emergency Department SITREP</b> , please specify numbers of patients below:                              |                                                                    |                                                                                   |
| Number of patients in the department                                                                        |                                                                    |                                                                                   |
| Number of patients waiting to be seen                                                                       |                                                                    |                                                                                   |
| Number of ATS 1 & 2 patients in the department                                                              |                                                                    |                                                                                   |
| Number of patients waiting for beds                                                                         |                                                                    |                                                                                   |
| Number of admitted patients with no bed allocated                                                           |                                                                    |                                                                                   |
| Number of admitted patients with bed allocated                                                              |                                                                    |                                                                                   |
| Number of admitted patients with bed allocated and bed ready                                                |                                                                    |                                                                                   |
| <b>Actions:</b>                                                                                             |                                                                    |                                                                                   |
| Ensure all ED BRAG Red and Black actions have been undertaken as per ED BRAG Action Cards                   | <input type="checkbox"/>                                           |                                                                                   |
| ED ANUM to call CAS to inform then that an ED Safety Alert has been sent out                                | <input type="checkbox"/>                                           |                                                                                   |
| Medical Director on-call informed <input type="checkbox"/> Notifying Consultant: _____                      |                                                                    |                                                                                   |
|                                                                                                             |                                                                    |                                                                                   |
| <b>Activate ED Full Capacity Protocol (weekdays 0800-1700)</b>                                              |                                                                    |                                                                                   |
| Executive approval for ED Full Capacity Protocol activation given by: _____                                 |                                                                    |                                                                                   |
| <b>Actions:</b>                                                                                             |                                                                    |                                                                                   |
| CAS to contact Helpdesk to prioritise bed cleans and Internal Logistics staff for ED Full Capacity Protocol | <input type="checkbox"/>                                           |                                                                                   |
| CAS to send out page to inform NUMs & Shift Coordinators that FCP has been activated                        | <input type="checkbox"/>                                           |                                                                                   |
| ED ANUM to liaise with CAS and ward NUMs to safely transfer patients to the wards                           | <input type="checkbox"/>                                           |                                                                                   |

### 3.4. Emergency Department Full Capacity Protocol (FCP)

The ED Safety Alert and Full Capacity Protocol (Figure 2) describes the escalation necessary when the ED (as the main point of entry for emergency admissions) has more patients than it can safely care for (ED overcrowding). This can result in an inability to provide safe, timely and efficient care to those patients and any subsequent patients who attend the ED.

The ED Full Capacity Protocol involves wards receiving an ED patient awaiting admission. Wards have the option to reconfigure their bed allocations by moving one patient who is pending discharge or, if the ED patient is deemed clinically suitable, into the corridor red taped area whilst the ward arranges an admission bed. The ED Full Capacity Protocol will only be instigated when the ED has reached BRAG status RED and are anticipating unsafe levels of patients awaiting admission and therefore sharing the admitting load through the organisation.

**FCP will be operationalized Monday – Sunday between the hours of 0600hrs - 1800hrs**

#### Criteria for activation:

- The Emergency Department BRAG status is Red – Exit Block
- The hospital escalation status is Code RED or a Code Yellow is likely to be declared within 12hrs based on bed capacity predictions and/or ambulance activity.

Should a ward feel unable to safely manage an additional patient they are to call the HC who will weigh up safety issues on the ward vs ED and determine whether an exemption is warranted.

When the ED Full Capacity Protocol has been activated, ward NUMs/HOOT are to be available to support the nursing team and ensure safety.

Staff must continue to undertake all care including CARE rounds on the FCP patient awaiting discharge. The FCP patient will be placed on Enterprise Bed Management in a 'Temp' bed and their BOSSnet records etc. maintained.

All FCP patients should be placed in the red taped area marked on the floor. The Shift Coordinator must ensure the Food Service staff are aware that the patient for discharge has been moved out of their room to ensure no patient is given the wrong meal.

#### Exclusion Criteria:

- No infection prevention concerns (patients on droplet precautions, airborne precautions, gastroenteritis/diarrhoea/vomiting, colonization with Candida Auris or Micro G alerts)
- No patients with an ADDS > 4 should be allocated to the red square at any time. If the patient who is due to be discharged deteriorates and is unable to be safely discharged, CAS must be contacted to ensure priority in finding a bed for the FCP patient even if on another ward.

### 3.5. Use of Over Census Beds

Over Census beds may be activated when the hospital is at BRAG status RED. The use of over census beds are to relieve overcrowding in the ED, create Intensive Care Unit (ICU)/Coronary Care Unit (CCU)/Acute Medical Unit (AMU) or Acute Surgical Unit (ASU) capacity, or place an unexpected urgent admission that would otherwise go to the ED.

There are a total of 11 over census beds at FSH in the towers and one at FH that can be used if required.

- It is the Ward Shift Coordinators responsibility to determine which patient on the ward is safest to receive care in the over census location. This should be a patient whose discharge is imminent and not a new patient from ED.
- Concerns about patient safety issues are to be directed to the Nursing Director (in hours) or the after-hours ALERT Clinical Nurse Specialist who will notify the Hospital Coordinator.
- Wards can accept one (1) patient over census at any time (24 hours/day, 7 days/week)

The following patients should not be considered as suitable for over census beds:

- Unstable
- Requiring cardiac monitoring/telemetry
- ICU admission
- Requiring high flow oxygen (home or long-term oxygen via nasal prongs/Hudson mask can be accommodated)
- BiPAP, CPAP or ventilated
- Requiring 1:1 nursing care
- Significant behavioural issues
- Require isolation room
- Patients with Glyceryl trinitrate (GTN), Salbutamol, Aminophylline, Amiodarone and Dopamine infusions are excluded
- ADDS score 4 and above
- Bariatric patients.

### 3.6.1. Location of Over Census Beds

#### Fiona Stanley Hospital

| Ward | Location                  | Specifics                              |
|------|---------------------------|----------------------------------------|
| 4A   | Treatment Room B-L-40317  | Fully equipped room, uses staff toilet |
| 4C   | TBC                       |                                        |
| 4D   | Bariatric Room 479 or 467 | Fully equipped room, no PES            |
| 5A   | Bariatric Room 504 or 517 | Fully equipped room, no PES            |
| 5B   | Bariatric Room 547 or 534 | Fully equipped room, no PES            |
| 5C   | TBC                       |                                        |
| 5D   | Bariatric Room 583 or 595 | Fully equipped room, no PES            |
| 6A   | Treatment Room B-L60325   | Fully equipped room, uses staff toilet |
| 6B   | Bariatric Room 45         | Fully equipped room, no PES            |
| 6C   | Bariatric Room 652 or 664 | Fully equipped room, no PES            |
| 6D   | Bariatric Room 81         | Fully equipped room, no PES            |
| 7A   | Bariatric Room 4 or 16    | Fully equipped room, no PES            |
| 7B   | Bariatric room 733 or 745 | Fully equipped room, no PES            |
| SSU  | Bed 91-100 in Day Stay    | Fully equipped area, no PES            |

#### Fremantle Hospital

| Ward | Location                                                                    | Specifics |
|------|-----------------------------------------------------------------------------|-----------|
| B9N  | 1 VEM bed available after 1400 but must be vacated by 1000hrs following day |           |
| B9S  | No over census available                                                    |           |
| B7N  | No over census available                                                    |           |
| B7S  | No over census available                                                    |           |



#### 4. Compliance/Performance Monitoring

Compliance against this policy will be evaluated by CAS using Western Australian Emergency Access Targets (WEAT) and Western Australian Elective Surgery Targets (WEST). Activation of the ED Full Capacity Protocol will be reported and reviewed at HEC.

#### 5. Related Policy Documents

[DoH Assertive Patient Flow and Bed Demand Management for Adult Services Policy and Guidelines 2011.](#)

[Department of Health Operational Directive OD 0445/13; Admission, Readmission, Services. FSFHG Management - Capacity and Access Management Policy and Procedure](#)

[FSFHG Bed Management - Discharge Policy and Procedure](#)

[FSH Transit Lounge Policy and Procedure](#)

#### 6. Related Standards

Standard 1: Governance for Safety and quality in Health Service Organizations

#### 7. Authorisation

| EXECUTIVE SPONSOR: |             |                     |                                    |             |              |
|--------------------|-------------|---------------------|------------------------------------|-------------|--------------|
| Version            | Date Issued | Compiled/Revised By | Committee/Consumer Group Consulted | Endorsed By | Revision due |
| 1                  | 04/2022     | ND CAS              | HEC                                | PLC         | 04/2025      |
| V1.1               | 03/2023     | Policy Team         | Review date smoothing              | HEC<br>PPC  | 04/2026      |

## Appendices

### 7.1. Appendix 2 – Capacity Management Action Cards

#### Action Card 1: ED ANUM Flow/Shift Coordinator

**Primary responsibility:** Management of ED staff and daily coordination of the shift including monitoring quality of care, patient flow, target compliance and liaison with internal services and external agencies

#### ACTIONS

##### GREEN STATUS

1. Undertake operational review of the department with the ED Medical Coordinator.
2. Monitor journey times and ensure that any patient not seen by a clinical decision maker (ENP/doctor) within 60 minutes of arrival or at the two and a half hour point with no plan in place is escalated to the ED Medical Coordinator.
3. Ensure ED medical rounds are following the ED SAFER Bundle.
4. Liaise with the CAS if there are obstructions to patient flow they may be able to assist with.
5. Ensure six hour plans/discharge plans for patients reaching the three hour point are developed in liaison with the Area Lead Consultant or Medical Coordinator.
6. Escalate to ED Medical Area Lead if a patient is not seen by specialty team within 30 minutes of referral to consider commencement of 6 hour plan.
7. Enter patient breach reason in EDIS for all patients who breach.
8. Review department BRAG status 2 hourly

##### AMBER STATUS

In addition to the above:

1. Move any patients pending discharge to the waiting room or Transit Lounge.
2. Ensure ESSU full and identify suitable patients for ESSU if required.
3. Review waiting patients and with Medical Coordinator, re stream any suitable patients to MDAC.
4. Ensure suitable patients are identified for Full Capacity Protocol

##### RED STATUS

In addition to the above:

1. Activate ED Capacity Escalation Plan in conjunction with the ED Medical Coordinator and take the described actions.
2. Ensure resuscitation capacity available at all times.

##### BLACK STATUS

In addition to the above:

1. Ensure clinical care is maintained by ED staff and specialty teams and that patients are appropriately nursed (e.g. on beds) whilst awaiting admission.

##### Code Yellow – Internal Emergency

In addition to the above:

1. Activate intentional rounding to ensure patient safety for all patients on a trolley for 4 hours or more, and for vulnerable patient

## Action Card 2: Nurse Director, Capacity and Access Service (in hours) Access Coordinator/HOOT CNM (out of hours)

**Primary responsibility:** Daily management of the patient flow across FSH and FH, coordinating adequate bed capacity and nursing/midwifery staffing levels

### ACTIONS:

#### GREEN STATUS

1. Collate bed status and staffing details from Service Nurse Directors/NUMs and specialist areas/departments.
2. Troubleshoot any pertinent capacity or staffing issues not able to be managed internally by each Service.
3. With NUMs/Service Nurse Directors, identify any constraints to discharge or transfer (via Red Green Day).
4. Provide an updated capacity status at any organization wide urgent status meetings.

#### AMBER STATUS

In addition to the above:

1. Ensure all possible solutions to potential breaches have been explored.
2. Identify available beds as soon as they become available and inform ED/AMU Coordinators for allocation by them.
3. Escalate potential breaches to relevant Nurse/Service Director and explain what actions have already been taken.

#### RED STATUS

In addition to the above:

1. Monitor with ED ANUM/Shift Coordinator any potential factors causing delays.
2. If criteria met, request permission to trigger ED FCP.
- 3.

#### BLACK STATUS

In addition to the above:

1. Liaise with Service ND to confirm that ED ANUM/Shift Coordinator has ensured that Consultant Leads have rounded and all patients have a clear plan.
2. In liaison with ED ANUM/Shift Coordinator and ED Medical Coordinator review SJA ambulance distribution.
3. Plan any additional capacity meetings.

#### Code Yellow – Internal Emergency

In addition to the above:

1. Liaise with ED regarding any need for additional nursing support to manage additional caseload and redeploy via other departments as available.
2. Request additional Internal Logistics/cleaning staff to support ED and ward discharges.
3. Undertake actions as outlined on Code Yellow Action Card.

## Action Card 3: Nurse Director (in hours) Hospital Coordinator (out of hours)

**Primary responsibility:** Tactical level daily management supporting the site operational teams to ensure adequate capacity and staffing across the organisation.

### ACTIONS:

#### GREEN STATUS

1. Troubleshoot any issues escalated by the ND CAS/Access/HOOT.
2. Forward escalation where needed to Service/Medical Director (in hours) Medical Director on call (out of hours).

#### AMBER STATUS

In addition to the above:

1. Maintain awareness of the evolving site activity and pre-empt any escalation in activity that could precede deterioration in performance.
2. Liaise with ND CAS/Access/HOOT to ensure all appropriate actions are being taken and that no further support is needed.

#### RED STATUS

In addition to the above:

1. Attend capacity meetings to support and direct actions to redress current situation and prevent further deterioration in status.
2. Ensure all areas have completed required actions as part of their BRAG action cards.
3. Review with ND CAS/Access/HOOT / HC any further remedial action that can be taken to maximize patient flow.
4. Ensure detailed and targeted communications messages have been issued internally or externally.
5. Escalate current status, predicted status and actions taken to ND CAS and Service/Medical Director (in hours) Medical Director on call (out of hours).

#### BLACK STATUS

In addition to the above:

1. Support the NUMs at the capacity meetings and ensure Service teams are clear about the actions to be taken as per BRAG Black action cards.
2. Inform ND CAS of any issues for escalation.
3. Participate in additional capacity meetings (the ND CAS will provide an updated site position).

#### Code Yellow – Internal Emergency

In addition to the above:

1. Attend Journey Board meetings to discuss any patients (SITREP category 1, 2 and 4 patients) for potential for discharge or transfer or who are 'red' under Red Green Day.
2. Liaise with Medical Directors to review each area's elective activity (including clinics and planned admissions).
3. Decide if any beds are to be opened and ensure appropriate staffing in place.
4. Undertake other actions as outlined on Code Yellow Action Card.

## Action Card 5: Service Director

**Primary responsibility:** To manage overall patient flow within their Service, focusing on future requirements including planning for future activity.

### ACTIONS:

#### GREEN STATUS

1. Ensure appropriate Service attendance at regular capacity meetings with provision of bed state predictions.
2. Ensure that there is appropriate allocation of staff to wards for optimal efficiency

#### AMBER STATUS

In addition to the above:

3. Ensure that tasks given at the capacity meetings are actioned within the Service and the ND CAS is updated on progress.

#### RED STATUS

In addition to the above:

1. In liaison with clinical leads/Consultants review all routine activity and/or elective admissions and identify any risks to normal service provision, taking action to mitigate where possible and highlighting any concerns to CAS.

#### BLACK STATUS

In addition to the above:

1. Communicate information on alert status and actions to staff within the Service.
2. Ensure accurate predictions on activity and Service capacity are provided to the ND CAS for any additional BRAG Black meetings, with details of mitigating internal actions taken.
3. Ensure sufficient staffing across the key Service areas to support increased activity and increase medical staffing support to ED and AMU where necessary.

#### Code Yellow – Internal Emergency

In addition to the above:

1. Liaise with Medical Directors to review each area's elective activity (including clinics and planned admissions).
2. Consider redeployment of staff to support areas of heightened activity.
3. Arrange for staff on non-clinical duties to support patient care or clinical service provision.
4. Review activity and resource implications over the coming 24 hours.
5. Undertake other actions as outlined on Code Yellow Action Card.

## Action Card 6: Medical Director

**Primary responsibility:** Strategic planning of Service activity to provide safe patient care and maintain adequate bed capacity and staffing. To provide clinical leadership for the Service.

### ACTIONS:

#### GREEN STATUS

1. Ensure awareness of site escalation status across Consultant teams.
2. Ensure medical rounds are following the SAFER principles with sick patients followed by those identified for discharge prioritized on the round.
3. Consider alternatives to inpatient care/early discharge at all times

#### AMBER STATUS

In addition to the above:

1. Liaise with HOS to update on site status.
2. Confirm senior staffing levels and address any gaps.

#### RED STATUS

In addition to the above:

1. Ensure daily senior review of all inpatients has occurred.
2. Liaise with HOS/Consultants to redeploy senior medical staff to clinical areas if needed.

#### BLACK STATUS

In addition to the above:

1. Ensure attendance at black status meetings.
2. Liaise with on-call Consultants to plan for adequate medical staffing for the on-call team.
3. Consider requesting via Director of Nursing and Midwifery/Director of Clinical Services/Executive Director activation of Full Capacity Protocol if ED compromised.

#### Code Yellow – Internal Emergency

In addition to the above:

1. Inform HOS/Consultant medical staff of capacity issues.
2. Review each area's elective activity (including clinics and planned admissions).
3. Instruct Consultants to work clinically to support AMU and ED.
4. Enforce strict DCS/MD only decision to admit in all specialties.
5. Ensure additional clinical rounds on wards involving senior medical and nursing staff.
6. Arrange additional medical staffing for on-call team if necessary or extended working hours.
7. Undertake other actions as outlined on Code Yellow Action Card.

## 7.2. Appendix 3 – Code Yellow Action Card

### Code Yellow – Internal Emergency

#### Capacity Escalation: Code Yellow Action Card

**Primary responsibility:** Management of patient flow Code Yellow to return the site to organizational function and ensure patient safety.

1. FSH executive team to meet as called by ND CAS and bring Service status and any issues to meeting.
2. Activate ED Full Capacity Protocol if not already actioned.
3. CAS to request Journey Board SITREP update by NUMs/Shift Coordinators
4. If not already completed, DCS to review ambulance distribution and discuss with SJA.
5. Executive to undertake review of planned admissions and theatre/procedural lists (including clinics).
6. ICU HOS to liaise with RGH ICU regarding the possibility of patient transfers.
7. Consider need for ICU Bed State Black or bypass for WCN or MH and CAS to notify other health sites of diversion status.
8. All direct admission requests to be approved by the DCS/ Medical Director.
9. All patients awaiting transfer to other health sites for >24 hours to be escalated to Executive Director
10. Consider expanded FH acceptance criteria to move appropriate patients to FH.
11. Staff and resource additional beds in SSU/DSU, SRS, ICU or FH.
12. Extend Transit Lounge opening hours if appropriate.
13. Service Directors to mobilise staff to support clinical areas from non-clinical activity and cancel non-essential meetings where necessary.
14. Service Directors to ask Consultants or Registrar to go to wards and do emergency discharge round.